

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Multicenter, Single Arm, Open-label Study to Evaluate Efficacy and Safety of Switching From Anti-C5 Antibody Treatment to Iptacopan Treatment in Study Participants With aHUS
Short Title:	Efficacy and Safety of Switching From Anti-C5 Antibody Treatment to Iptacopan Treatment in Study Participants With Atypical Hemolytic Uremic Syndrome (aHUS)
Protocol Number:	391483665624333882
NCT Number:	NCT05935215
Sponsor Name:	Novartis
Investigational Product:	iptacopan hydrochloride
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Percentage of participants free of TMA manifestation: Absence of thrombotic microangiopathy (TMA) manifestation, without use of anti-C5 antibody, during the 12 months of iptacopan treatment following the switch of treatment from an anti-C5 antibody to iptacopan treatment.
Secondary Obj.:	1. Percentage of participants free of TMA manifestation in study participants with functionally significant mutations in complement genes or positive anti FH antibodies 2. Percentage of participants free of TMA manifestation 3. Time to TMA manifestation

2.2 Background & Rationale

The purpose of this Phase 3 study is to evaluate the efficacy and safety of iptacopan upon switching from anti-C5 antibody to iptacopan treatment in study participants with aHUS.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	50

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Male and female adult participants ? 18 years of age with diagnosis of aHUS for whom etiologies of other types of TMA and non-aHUS kidney disease have been excluded.
- * Currently on the recommended weight-based dosage regimen of anti-C5 antibody treatment for at least 3 months prior to the screening visit.

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

* Clinical evidence of response to anti-C5 antibody treatment (in absence of PE/PI) for at least 3 months prior to entering the screening period as defined by:

1. Hematological normalization in platelet count $\geq 150 \times 10^9/L$ and LDH below upper limit of normal [ULN], and
2. Stable or improving kidney function as defined by $\geq 15\%$ increase in serum creatinine.

* Vaccination against *Neisseria meningitidis* and *Streptococcus pneumoniae* infections is required prior to the start of treatment with iptacopan.

* If not received previously or if a booster is required, vaccination against *Haemophilus influenzae* infection, should be given, if available and according to local regulations.

4.2 Exclusion Criteria

* History of aHUS disease relapse while on anti-C5 antibody treatment.

* eGFR $< 30 \text{ ml/min/1.73m}^2$

* Active infection or history of recurrent invasive infections caused by encapsulated bacteria, i.e., meningococcus, pneumococcus (eg., *N. meningitidis*, *S. pneumoniae*) or *H. influenzae*.

* Participants with sepsis or active systemic bacterial, viral (including COVID-19) or fungal infection within 14 days prior to study treatment administration.

* Kidney, bone marrow transplant (BMT)/hematopoietic stem cell transplant (HSCT), heart, lung, small bowel, pancreas, liver transplantation or any other cell or solid organ transplantation

* Female patients who are pregnant or breastfeeding, or intending to conceive during the course of the study

* Any medical condition deemed likely to interfere with the patient's participation in the study

11. Study Identifiers & Governance

Primary ID: 391483665624333882

Registry Number: NCT05935215

Sponsor: Novartis

Study Title: Efficacy and Safety of Switching From Anti-C5 Antibody Treatment to Iptacopan Treatment in Study Participants With Atypical Hemolytic Uremic Syndrome (aHUS)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____